A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 April 2026

Dr Monika Trivedi

Dear Dr Trivedi,

Re: Ms Victoria Carstairs
DOB: 1983-09-29
Address: Unit 5, 169-173, SHEPPARTON, VIC, 3630

Summary

- **Chronic abdominal pain, bloating, and early satiety likely related to visceral hypersensitivity and carbohydrate malabsorption.**
- **Bowel habits suggestive of pelvic floor dyssynergia on a background of chronic constipation with overflow diarrhoea.**
- **For gastroscopy and colonoscopy to exclude organic pathology given family history and change in weight.**
- **Implementation of dietary modifications including cessation of chewing gum and artificial sweeteners.**
- **Referral to St Vincent's Public Hospital for endoscopic evaluation.**
- **No action required by you.**

I had the pleasure of conducting a telehealth appointment with Victoria, thank you for referring her.

Symptoms and history of presenting complaint

Victoria has a protracted history of gastrointestinal symptoms dating back to her mid-twenties. Her symptoms are multifaceted, consisting of chronic abdominal pain, significant bloating, and early satiety. She describes the pain as being present in the upper, middle, and lower abdominal regions, often triggered by dairy, caffeine, and acute stress. She does not feel that passing wind or opening her bowels provides significant relief from the pain or the persistent sense of fullness.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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In terms of her bowel habits, Victoria predominantly experiences constipation, opening her bowels approximately every three days. These movements require significant straining and are associated with a sense of incomplete evacuation. Intermittently, she experiences episodes of diarrhoea, occurring roughly once a week, which have been associated with faecal incontinence. She also noted a history of urinary urgency and frequency, although this has marginally improved recently.

Of note, Victoria has experienced weight loss, moving from 70 kilogrammes last year to a lower weight currently. She also reports occasional symptoms of heartburn and regurgitation, particularly when her bloating is at its most severe.

Diet

Victoria's diet is currently elevated in FODMAP content, particularly through her consumption of specific fruits. Her caffeine intake is negligible as she avoids coffee and tea due to their stimulating effects. She does not consume alcohol. However, her diet includes regular consumption of soft drinks (including Pepsi Max) and cordial, both of which contain artificial sweeteners. She also regularly uses chewing gum.

Past medical history and medications

Victoria’s medical history is significant for endometriosis and fibroids, the latter of which are currently being monitored. She underwent a cholecystectomy approximately ten years ago. She also has a history of asthma. Her regular medications include paracetamol for pain management and Ventolin as required.

Treatments trialled before and investigations performed

Victoria previously trialled a low FODMAP diet, which she found significantly improved her symptoms of pain and urgency. However, she was unable to maintain the restrictive nature of the diet long-term. She has not previously undergone endoscopic evaluation.

Social history, family history

Victoria lives with her 11-year-old son. She is currently not working. She was very candid regarding her social situation, noting that her symptoms originally began during a period of significant domestic stress in a previous relationship.

Examination

I was unable to examine Victoria as this was a telehealth appointment.

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Impression and plan

My impression is that Victoria's symptoms are driven by a complex interplay of three factors: carbohydrate malabsorption (specifically related to FODMAPs and artificial sweeteners), visceral hypersensitivity, and pelvic floor dyssynergia. The latter likely explains her symptoms of straining, incomplete evacuation, and associated bladder urgency.

Given the duration of her symptoms, and her recent weight loss, I believe it is prudent to proceed with a gastroscopy and colonoscopy. I have discussed the risks of these procedures, including bleeding, perforation, and anaesthetic risks, and she is keen to proceed.

Logistically, Victoria does not have private health insurance. I have therefore organised for her to be placed on the waiting list at St Vincent's Public Hospital as a non-urgent case. We discussed the requirements for an adult escort post-procedure, and I will liaise with the hospital regarding her specific social circumstances to see if an exemption or short admission can be facilitated.

In the interim, I have recommended the following:

1. Cessation of chewing gum and artificial sweeteners (found in diet soft drinks and cordial), as these are known triggers for bloating and loose stools.
2. Switching to regular sugar-based soft drinks if she chooses to consume them.
3. Regular consumption of kiwifruit to help regulate her bowel movements.

I will review her again following her investigations.

Thank you again for referring this patient.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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